

Sexual and Reproductive Health and Associated Challenges in Iranian Men: A Qualitative Study

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Abstract

Men, like women, experience a wide range of serious health issues, including sexual and reproductive health concerns, which must be addressed alongside women's health issues to improve the health of societies. This study aimed to explore Iranian men's perceptions of sexual and reproductive health as well as associated challenges. This qualitative study was designed through the conventional content analysis approach. Purposive sampling with maximum variation was employed to select 26 participants, and data were collected through in-depth, semi-structured interviews. Data analysis was performed using the conventional qualitative content analysis method proposed by Zhang and Wildemuth. Analysis of the interview transcripts revealed that men's perceptions of sexual and reproductive health and relevant challenges were classified into two categories and five subcategories. These included "socio-economic security and unmet needs" (consisting of three subcategories, i.e., "mental health," "lifestyle," and "sexual health") and "cultural-religious factors and unmet needs" (consisting of two subcategories, i.e., "Seeking for health and its barriers" and "religious norms"). According to the research findings, numerous sexual and reproductive health concerns and challenges are perceived by Iranian men. Exploring these concerns and challenges can help provide an in-depth insight into their needs for optimal care, empower them through education, and obtain support from family and society. This can improve men's reproductive health.

Keywords

sexual health, reproductive health, men, challenges, concerns, Iran, men's health, behavior

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Background

The World Health Organization (WHO) and the United Nations Fund for Population Activities (UNFPA) define reproductive health as "a state of physical, mental, and social well-being and not merely the absence of disease or infirmity in all aspects of the reproductive system as well as its functions and processes" (Canton, 2021).

Men, like women, experience a wide range of serious health issues, including sexual and reproductive health concerns, which must be addressed alongside women's health issues to improve the health of societies (Baker & Shand, 2017). It has received less attention than women's sexual and reproductive health; therefore, there are global concerns about this issue (De Jonge & Barratt, 2019). In

most societies, there are no structured programs or dedicated services to men's sexual and reproductive health (Amoo et al., 2017; Gottert et al., 2025; Weerasinghe et al., 2025). As a result, men have limited access to sexual and reproductive health services and insufficient information regarding their availability and utilization (Bersamin et al., 2017; Gottert et al., 2025; Weerasinghe et al., 2025). Iranian men also have many unmet needs related to sexual and reproductive health (Rahimi-Naghani et al., 2016). Therefore, establishing specialized and accessible reproductive health care centers for men is essential, particularly in low-income countries (Gottert et al., 2025; Ruane-McAteer et al., 2019; Weerasinghe et al., 2025). These centers should provide high-quality reproductive health services for all men and be able to



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meet their needs in this regard (Gottert et al., 2025; Ruane-McAteer et al., 2019). The improvement of men's sexual and reproductive health not only enhances their emotional, mental, and physical health but also has significant effects on family health, that is, the health of women and children, which in turn reduces the rate of alcohol abuse and violence (Mataraarachchi et al., 2025). Moreover, the welfare index and quality of life in men have direct correlations with their reproductive and sexual health (Uka et al., 2024).

Men's sexual and reproductive health is influenced by various factors such as sociodemographic characteristics, lifestyle factors, biomedical factors, and environmental-behavioral factors (e.g., exposure to environmental toxins, electromagnetic radiation, heat) (Cai et al., 2025; Maluin et al., 2021; Mohammadzadeh et al., 2024; Salas-Huetos et al., 2019; Tian et al., 2024). In addition, it is also affected by social norms, cultural and religious norms, mass media, and governmental policies on the health system; thus, it is essential to analyze these factors to improve men's sexual and reproductive health (Agyei et al., 2025; Ashriady et al., 2024; Lirios et al., 2024).

Given the diversity of geographic, environmental, socio-cultural, and socioeconomic conditions in different countries, the health-related needs of different groups should locally be assessed in different countries. Review studies have shown the importance of men's involvement in reproductive health research including qualitative studies (Gottert et al., 2025; Ruane-McAteer et al., 2019). Findings from such studies can inform health plan development, allocate resources and budget, and set regional research priorities (Gottert et al., 2025; Ruane-McAteer et al., 2019). There is apparently a paucity of information on the comprehensive perception that highlights Iranian men's perspectives about sexual and reproductive health and relevant challenges. Qualitative studies can be efficient in assessing deep and personal concepts such as reproductive health. Such studies also help realize human phenomena by emphasizing the social contexts (Speziale et al., 2011). Therefore, this qualitative study was conducted to analyze Iranian men's

perceptions regarding the sexual and reproductive health issue and relevant challenges. This study constituted the first phase of a sequential exploratory mixed-methods design aimed at developing and psychometrically evaluating an assessment tool for reproductive health-related behaviors among Iranian men. Recognizing sexual and reproductive health challenges among Iranian men can provide the basis for designing future interventions and policy reforms. This can also help enhance men's sexual and reproductive health by empowering them through education and obtain support from family and society.

Method

Study Design and Participants

This study represents the qualitative phase of a sequential exploratory mixed-methods research project aimed at developing and psychometrically evaluating an assessment tool for reproductive health-related behaviors among Iranian men. The study protocol has been previously published (Geranmayeh et al., 2020). The participants consisted of 26 men who were recruited based on purposeful sampling strategy based on the following predefined criteria: be the literate men living in Tehran, aged 18 to 60 years with no infertility problems (because men with infertility problems may have different perceptions of reproductive health). Different perceptions were achieved with maximum variation strategy in terms of age, education, occupation, marital status, economic status, number of children, and region of residence.

Setting

This study was conducted in Tehran, the capital city of Iran. Iran is an ethnically diverse country and has experienced reductions in its gross domestic product (GDP) growth due to international sanctions in recent years. Economic sanctions have had negative impacts on the public health in Iran by impairing social determinants of

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health and access to medication and care. These sanctions widen economic inequality and health gap (Aloosh et al., 2019). With 13 million inhabitants including 3.7 million men, Tehran is the most populous city in Iran.

Data Collection

Data were collected from June 2019 to February 2020. Ethical approval for this study was obtained from the Ethics Committee of Tehran University of Medical Sciences. In addition, the necessary permissions were granted by the Health Deputy of Tehran University of Medical Sciences to conduct the research at the health centers. In-depth, semi-structured interviews were conducted in this study to collect data. In-depth interviews are efficient in exploring interviewees' perceptions and experiences (Speziale et al., 2011). The corresponding author conducted all interviews in Persian; the transcripts were subsequently translated into English. All interviews were conducted face-to-face at times and locations chosen by the participants. Moreover, private rooms in health centers affiliated with Tehran University of Medical Sciences were used to ensure accessibility, comfort, and privacy. Two initial interviews were conducted in a pilot research plan to familiarize the researcher with possible and unpredicted problems as well as the arrangement of interview questions. Prior to each interview, the participants were provided with the necessary information regarding the nature and goal of the study. They were also ensured the confidentiality of their interviews and asked for written consent for participating in the study and recording their interviews. On average, each interview lasted for nearly 45 to 60 min. Sampling was continued until data saturation was achieved—that is, no new pieces of information or codes were found in the data—in the 26th interview. The interview guide was developed broadly by the research team to query participants' perception of reproductive health and relevant challenges (the appendix). Interviews began with broad, open-ended questions, followed by probing questions to elicit deeper and more detailed responses. Some of the questions were as follows: "Could you share your thoughts about reproductive health?" "Would you tell me how you feel about your reproductive health?" "What factors do you believe influence men's reproductive health?" "What behavior do you think affects your reproductive health?" "What actions do you take to improve your reproductive health?" and "What resources or services would help you access information and care related to reproductive health?" For the clarification of explanations, further probing questions were then asked ("Could you give me an example?" or "Would you explain more?"). During the interviews, the researcher documented nonverbal data including

participants' tones, facial expressions, and posture, as well as the interview time and location on a special sheet.

Data Analysis

Data management was performed in MAXQDA 10. After each interview, data were analyzed through a conventional content analysis approach, proposed by Zhang and Wildemuth (2009). The advantage of a conventional approach to qualitative content analysis, as in this study, is the ability to collect data directly from participants without using preconceived categories and previous theoretical perspectives. In this method, the findings are grounded in participants' perspectives and rooted in the textual data; the knowledge generated from content analysis is based on the unique views of participants and is rooted in the text data (Mayring, 2021). All interviews were audio-recorded and transcribed verbatim. The transcribed interviews were read several times to identify the semantic units related to reproductive health. The codes detected on the basis of semantic units resulted from participants' narratives. The codes were then classified as categories and subcategories in terms of differences and similarities. Table 1 shows the content analysis examples, encoding processes, categories, and subcategories.

Rigor and Trustworthiness

In this study, to ensure the acceptability of the data collected in the qualitative phase, the transcripts of interviews were reviewed by a number of participants (member check), revised by the supervisors (external check and peer debriefing), and continuously evaluated by the researcher (prolonged engagement), and purposive sampling was performed with maximum variety. Also, the interviews were conducted in Persian and then translated into English for publication by two independent bilingual translators. In addition, the translated versions were then reviewed and approved by a third independent reviewer to ensure semantic equivalence. To ensure the research reliability, the results were provided for an external supervisor to conduct the research audit. The transcripts of interviews and codes were reviewed by the research team members, and the external supervisor to verify the research findings. Moreover, to enhance transferability in this study, the samples were selected with maximum diversity in terms of age, education, and so on. The research accuracy was confirmed using the researchers' credibility, that is, reviewing and revising the interviews, codes and categories extracted by the research team members, long-term interaction with the participants, and maintaining continuous communication with the corresponding author's notes.

Table 1. An Example of Analysis Process.

Meaning unit	Code	Subcategory	Main category
- The problem is that if there is any service since it is provided by a female staff, I am not comfortable. In general, major changes are needed to provide men-specific services in health centers.	Lack of male staff in health centers to receive services	Seeking for health and its barriers	Cultural-religious factors and unmet needs
- Since there is not any specific person or room for men or it is completely formal and they do not ask about sexual and reproductive issues, so I may be ashamed to ask my question.	Lack of motivation in the female health staffs to provide reproductive health services to men		
- The problem is that if there is any service since it is provided by a female staff, I am not comfortable. In general, major changes are needed to provide men-specific services in health centers.	Lack of a proper source of information due to taboo issues related to reproductive health		
- There are many times when I encounter a question or problem about reproductive and sexual health. But there is no suitable source to solve the problem in Iran due to the taboo of discussing reproductive health issues. There is no suitable program in this field on TV. I cannot find reliable information in this field in cyberspace, and not only can my problem not be solved, but it may double.			

Results

Twenty-six participants (26 men) participated in this study. Due to the paucity of information and time, nine people refused to participate in the study before recruitment. The participants were aged between 22 and 57 years with a mean age of 39.30 ($\pm$ 8.17). The demographic characteristics of the participants are presented in Table 2. After the interview transcripts were analyzed, men's perceptions of sexual and reproductive health and relevant challenges (560 codes extracted from the interview texts) were organized into two main categories and five subcategories: "socio-economic security and unmet needs" (consisting of three subcategories, i.e., "mental health," "lifestyle," and "sexual health") and "cultural-religious factors and unmet needs" (consisting of two subcategories, i.e., "Seeking for health and its barriers" and "religious norms") (Table 3).

Socioeconomic Security and Unmet Needs

All participants emphasized the significant impact of socioeconomic factors—such as financial hardship, employment status, and perceived social injustice—on men's sexual and reproductive health. Several participants associated sexual dysfunctions, including decreased sexual desire, erectile dysfunction, premature ejaculation, and compulsive masturbation, with economic stressors and work-related psychological pressures. They also highlighted that social inequities influence men's access to reproductive health information and services, particularly in low-income urban areas. According to the participants, current socioeconomic challenges have substantially affected their mental well-being, lifestyle patterns, and, consequently, their sexual and reproductive health.

Mental Health. From the perspective of the participants, men's sexual and reproductive health are strongly affected by their moods and mental states. Mental health problems such as anxiety, depression, fatigue, low self-esteem, and history of sexual abuse can affect men's sexual desire and function. Most participants attributed these mental health challenges to economic and occupational insecurity, job-related stress, unfavorable workplace conditions, limited life skills, family-related difficulties, and insufficient social and familial support. In other words, participants believed that men's mental health problems caused by economic and workplace problems could also involve their partners and children at home. This may cause family conflicts which might consequently lead to divorce and family breakdown. Participants also believed that sexual disorders caused by mental health problems such as premature ejaculation, erectile dysfunction, and reduced sexual desire might increase the chance of high-risk behavior including use of psychedelic drugs, medicine abuse, alcohol, and drug abuse. Some also indicated that depression and anxiety may elevate the risk of suicidal ideation. Men may also be addicted to excessive masturbation when they are under psychological pressures.

A participant said,

Given the current situation, I do not think about sexual matters at all. Inflation and price increases on the one hand and rents and fear of eviction on the other hand have made me tired and frustrated. I have to work day and night to provide the basic necessities for my family. (P 20)

Another participant stated,

Although my financial situation isn't bad, I'm sometimes under pressure. For example, covering the expenses and paying the

Table 2. Demographic Information of the Participants (N = 26).

Characteristic	Mean (SD) or Number (Percent)
Age (<i>M</i> ± <i>SD</i>)	38.30 ± 8.17
Education level (<i>N</i> , %)	
Basic literacy	1 (3.84)
Diploma	8 (30.76)
University	17 (65.38)
Occupation (<i>N</i> , %)	
Employed	23 (88.46)
Unemployed	3 (11.54)
Socioeconomic status (<i>N</i> , %)	
Good	2 (7.70)
Average	19 (73.07)
Weak	5 (19.23)
Marital status (<i>N</i> , %)	
Married	24 (92.30)
Single	2 (7.70)
Marital duration (<i>M</i> ± <i>SD</i>)	11.62 ± 2.1
Number of children (<i>M</i> ± <i>SD</i>)	1.81 ± 0.85
Place of living	
City	25 (96.15)
Village	1 (3.84)

Table 3. Main Categories and Subcategories Derived in This Study.

Main categories	Subcategories
1. Socioeconomic security and unmet needs	Mental health Life style Sexual health
2. Cultural-religious factors and unmet needs	Seeking for health and its barriers Religious norms

debts put me under psychological pressure. At such times, I suffer from premature ejaculation and feel worthless and ashamed. So, I often avoid sexual intimacy. (P 15)

A participant talked about the importance of job security and well-being, "Working conditions are very important. I get very tired during the day, and my desire for sex has declined. My problems might be solved if there is more support from my employer" (P 11).

Another participant complained about workplace-related pressure and interpersonal conflict: "The workplace-related problems and conflicts with colleagues, employers, and clients never end. When I return home and end up arguing with my wife, I no longer have any sexual desire" (P 22).

Another participant noted,

I should work my whole life for nothing. There will be no progress in our life because we have a fixed income regardless

of how much we work, and prices are rising dramatically and non-stop. Under such conditions, nobody likes to think about reproductive issues and having children. (P 23)

A participant complained about frustration and sadness due to lack of psychological support for retired employees from the government and society,

Retirement has no true meaning in Iran. When a person is retired, he should enjoy this period but, on the contrary, I feel rejected, humiliated, and depressed. I now face greater financial and livelihood challenges than when I was employed. There is no active support organization or group for the retired people in society. All of these issues affect people's sexual and reproductive health. (P 21)

Concerning the mental health problems, a participant said,

Men's psychological problems affect their sexual function and self-confidence. I have so many thoughts, fantasies, and problems that I have developed erectile dysfunction and sometimes I have to resort to taking medication and even drugs, which is completely undesired for me. (P 6)

Another participant talked about lack of life skills education and its psychological consequences,

We haven't been taught in childhood how to live. We immediately get aggressive for nothing and then regret it, which might be too late. The majority of familial and occupational problems are due to the fact that we don't know how to solve our problems and how to control our anger. (P 8)

Another participant stated,

I think the main problem is that we don't know how to communicate at all. In fact, we haven't learned it. The right communication skills help people have the right peace of mind and solve their problems properly. This sense of relaxation can have a positive influence positive effect on the quality of men's sexual relationships and even their fertility issues. (P 24)

Lifestyle. Most of the participants reported unhealthy lifestyle patterns, including skipping breakfast, insufficient sleep, and eating fast food regularly. They believed that men's lifestyle would have a significant impact on their general health including their reproductive health; however, lifestyle would be strongly influenced by socioeconomic status. Poor social infrastructure, especially in poor communities, causes unhealthy lifestyles among men. Conditions such as economic hardship, occupational instability, heavy traffic, long working hours, and working in a standing position for a long period of time cause fatigue and insomnia. These problems decrease sexual satisfaction, increase sexual dysfunctions such as

premature ejaculation and erectile dysfunction, and potentially contribute to reproductive problems.

A participant stated,

It is now very common in metropolises like Tehran. Access to healthy food is limited, and I may only eat a proper home-cooked meal once a week. I eat fast food all the time. Even if I wanted to, even if I wanted to eat healthier, my long working hours outside the home make it difficult. (P 13)

Another participant added,

Everyone wants to live in a decent neighborhood, but housing prices in Tehran are extremely high. I have to rent in a marginal area far from my workplace. By the time I get home, it's very late, and I'm too exhausted to prepare a proper meal. I also have to wake up early the next morning. As a result, intimacy between couples decreases. (P 18)

Another participant talked about his concerns about the effects of occupational conditions on reproductive health: "Because of my work conditions, I have to stand most of the time, so I have developed varicocele. I had a surgery but still have trouble getting my partner pregnant" (P 16).

Sexual Health. According to the participants, men need to know more about the physiological differences between men and women in sexual matters, prevention and treatment of sexually transmitted diseases, and improving their reproductive health. However, most of the participants stated that the medical staff could be the best source of information. They also stated that they were not willing to talk about their reproductive health problems to their friends. Nevertheless, the participants were not satisfied with the accessibility and quality of sources of reproductive health information in society. Participants also believed that worsening economic conditions in Iran—particularly over the past decade—have delayed marriage among young people, potentially increasing the likelihood of high-risk sexual behaviors.

A participant talked about the importance of identifying physiological differences between men and women for their sexual health: "While women can immediately have another orgasm, men need a few hours and even a day. Both men and women must understand these differences; otherwise, they may experience anxiety" (P 7).

Another participant talked about the importance of awareness of sexually transmitted infections:

A friend of mine had genital warts and had no idea whether it was a wart or something else. It is interesting that although I explained it to him, he still didn't think it was important. He didn't know how it is transmitted. It is very important to seek information and solve these problems. (P 24)

A participant stated,

I'm not eager to discuss my sexual and reproductive problems with those close to me, especially with my friends, because I am not sure if they are trustworthy. However, the cultural and health contexts of Iran inhibit people from visiting a health center for counseling with an expert comfortably. I think it is essential to have a department dedicated to sexual and reproductive health problems with both male and female staff so both men and women feel comfortable to seek solutions to their problems. (P 14)

Another participant attributed unwillingness to use medical services among men to the provision of undesired services and irresponsibility of medical staff, "The provided services are rarely planned for men, particularly in relation to sexual and reproductive issues. The medical staff is not accountable at all when it comes to men" (P 6).

Cultural and Religious Factors and Unmet Needs

The participants had contradictory opinions on the roles of culture and religion in reproductive health. Some participants introduced religious and cultural norms of Iran's society as an inhibitor of high-risk sexual behavior and believed that religious notions and cultural norms could be the basis of a healthy sexual and reproductive life. However, others believed that cultural and religious norms would act as a barrier to their reproductive health and access to services. They also believed that the taboo nature of discussing sexual topics and prohibition of premarital sexual relationships would provide grounds for ignorance and risky behavior.

Seeking for Health and Its Barriers. The participants stated that despite all problems that affected their reproductive health, they were willing to seek reproductive health services and sex education to improve their sexual and reproductive health. However, due to the taboo nature of this issue in Iran, there is practically no education and comprehensive quality plans for services related to sexual and reproductive health in men. Moreover, most of them complained about unaccountable medical staff, lack of same-sex staff to provide services specific to men, and lack of proper sources of information. Some other participants mentioned individual factors such as feeling ashamed of talking about reproductive health issues as a barrier to the attainment of education and counseling. They attributed it to the lack of same-sex staff to provide such services in medical centers.

A participant stated,

One reason that I am personally not interested in visiting a medical center is that most of them are dedicated to women and children. I went there a few times but mostly for bureaucratic

purposes. Since there is no specific person or room for men or as there is a completely formal atmosphere in which they do not ask about sexual and reproductive issues, I might feel ashamed of asking my questions. (P 26)

Another participant stated, “The problem is that if there is any service, since it is provided by a female staff, I am not comfortable. In general, major changes are needed to provide men-specific services in health centers” (P 2).

A participant said,

There are many occasions when I have a question or problem about reproductive and sexual health, but there is no suitable source to solve the problem in Iran due to the taboo of discussing reproductive health issues. There is also no suitable program on TV. I cannot find reliable information in the cyberspace, so I’m unable to solve my problem, and it may get even worse. (P 16)

A participant stated,

I had many questions and problems during puberty, but I did not dare to ask my parents. All my information was from friends of my age and older, while they, like me, did not have the right information. There should be specific centers in the community to provide these services so that people’s health, especially that of adolescents, is not endangered. (P 13)

Another participant mentioned,

If there is a proper center or even a source of information, it is wonderful. Unfortunately, there is not adequate education about sexual and reproductive health problems in Iran. These educations should be provided in an age-appropriate manner from the early days of puberty. If there is a problem, where should a man go to? If the problem remains unsolved, it may have irreparable consequences. (P 17)

Another participant stated,

Cyberspace, including various Telegram channels, can provide people with incorrect information, so it has very destructive effects on men’s sexual and reproductive health. Unlike the national media, which unfortunately does not have any program in this field, the government does not control the content of cyberspace programs. (P 23)

According to another participant, “Generally, talking about sexual matters and issues would be regarded as inappropriate behavior, and a person with such questions prefer not to ask them at all or solve the problem because it may bring disgrace” (P 3).

Another participant talked about the role of cultural norms,

In our society, only married men and women can live together. Cohabitation is against traditions and the culture of Iranian society. At the same time, sex education is a taboo subject in Iran. These factors increase the chance of marriage failure because the youth have almost no knowledge of the matter. (P 8)

Religious Norms. According to the participants, religious notions in Iranian culture have both positive and negative effects on reproductive and sexual health. Proper interpretation of these notions to men can help solve their problems.

With respect to connection with God, a participant stated,

For example, whenever I have negative thoughts for high-risk behavior, I distract myself to something else. Connection with God is an important factor. It can absolutely calm your unrest mind. This has nothing to do with religion. A person may not strictly follow religion but can still establish a meaningful connection with God that a religious person is not able to have. That you say “I place myself in Your hands” protects me from bad things and the evil. It is an important exemplar of protective behavior. (P 24)

However, some participants mentioned the negative aspects of religion as a barrier to expressing feelings, emotions, and intimacy between the couples and also as an important factor in having a sense of guilt and other relevant psychological problems in connection with sexual problems.

A participant talked about the effect of religious beliefs on the sense of guilt and regret:

Sexual relationships outside marriage is a sin in our religion, but it may happen to the youth who cannot get married. It also applies to masturbation, but nearly all the youth may experience it and may feel guilty afterwards. A persistent feeling of guilt can be destructive. (P 13)

Another participant recounted, “My wife grew up in a religious family and is not comfortable to have a sexual relationship with me, as her husband. She sometimes avoids intimacy because she associates it with sin” (P 23).

Discussion

This study was conducted in Iran to analyze men’s perceptions of sexual and reproductive health and related challenges. The results of this study were used for design and psychometrics of the reproductive health-related behaviors assessment tools among Iranian men. The results indicated that men’s perceptions of sexual and reproductive health are shaped by the traditional sociocultural context of Iranian society. In addition, Iran’s socioeconomic

problems have changed their perception of sexual and reproductive health. However, men believed that they could achieve sexual and reproductive health by practicing healthy behavior and avoiding high-risk behavior through religious beliefs and health-seeking behaviors. Similar patterns have been reported in studies of other low-income countries. A systematic review of the sociocultural underpinnings of men's sexual and reproductive health in the Middle East and North Africa region has found that cultural and religious issues have a profound impact on sexual and reproductive health and related seeking behaviors among men living in the Middle East and North Africa. Therefore, to promote sexual and reproductive health in men, health policymakers and health care providers in these countries should focus on the cultural, religious, and social issues of their society that can affect men's sexual and reproductive health (El Ansari et al., 2025).

The results also showed that there is many challenges in relation to male sexual and reproductive health from men's standpoint. However, most of the participants mentioned socioeconomic and cultural norms, concerns, and pressures such as the taboo subject of sexuality, poor occupational status, low income, and work-related stress as a foundation of other sexual and reproductive health problems. In addition, economic conditions and inflation may negatively affect Iranian women's sexual satisfaction. Women's dissatisfaction is likely due to their husband's job and insufficient income, and men's unfavorable and stressful working conditions, which negatively affect their sexual relationships (Afzali et al., 2020). These problems not only have direct, adverse impacts on men's sexual and reproductive health but also affect their access to medical and welfare services in society. The participants mentioned the lack of proper education in adolescence, lack of social support, and lack of life skills education as important factors of social and mental harms, which would affect their sexual and reproductive health. The inability to control anger in social and work interactions could cause psychological and mental pressure in men and put their sexual and reproductive health at risk. Other studies have analyzed the relationship between mental health and sexual-reproductive health (Duby et al., 2021; Vanderkruijck et al., 2021; Zaks et al., 2023). Unhealthy lifestyles, especially in crowded and expensive cities where working conditions prevent Iranian men from exercising and eating a healthy diet, along with physical illnesses, were among the major issues that had adverse effects on men's sexual and reproductive health. This finding likely stems from the negative effects of overweight and obesity on individuals' sexual and reproductive health (Peel et al., 2025). Also, the participants believed that seeking information and services concerning reproductive health from proper sources could greatly improve their reproductive health. However, most participants complained about the lack of

related information sources such as the national media, the lack of good service, and the irresponsibility of medical staff for men. To confirm this, a cross-sectional study was conducted in Iran to analyze the unmet needs related to reproductive health. It showed that the need for information and counseling services for men was as necessary as the need for clinical services (Javadnoori et al., 2018). Based on the results of a qualitative study in which five focus group interviews were conducted with 22 midwives, midwives believed that although male reproductive health services enhance the general health of the community, no one in health centers has taken responsibility for it (Grandahl et al., 2019). As a result, unmet needs in sexual and reproductive education for Iranian men are another important challenge related to their sexual and reproductive health. One reason behind unmet needs perceived by men in this regard is that sexual and reproductive education is a taboo subject in Iran. Previous studies have likewise documented the negative implications of such taboos for sexual functioning and psychological well-being (Karimian et al., 2018). Due to cultural beliefs regarding sexual problems as a taboo subject, most of the participants believed that their primary sources of information were friends and websites, which may be unreliable. The results of a qualitative study in Iran also showed that Iranian men have limited information about sexual and reproductive problems that they obtain from their friends (Lotfi et al., 2022). According to the results of a qualitative study in Iran, men's sexual and reproductive education is restricted by personal and structural problems and sociocultural taboos (MirzaiiNajmabadi et al., 2019). Since it is a taboo subject, the medical staff is not willing to share such information. This is an issue in the field of sexual and reproductive health not only in Iran but also in other countries. The results of a study in Denmark showed that sex education is a taboo topic in Danish health centers. The results of this study showed that health care providers were not very interested in responding to clients with sexual problems because sexuality was a sensitive and taboo topic in the Danish health care system, which led to client dissatisfaction (Traumer et al., 2019). To enhance men's sexual and reproductive health, these barriers should be eliminated by increasing men's knowledge and taking necessary measures to overcome such sociocultural taboos. Motivating medical staff to provide male sexual and reproductive health services could be an important measure (O'Connor et al., 2019).

According to the participants, since the majority of them lack enough time for sexual and reproductive health education, the health care providers can provide their wives or sexual partners with relevant programs to alleviate men's sexual and reproductive health problems. Wives play an important role in resolving men's sexual and reproductive health problems. Therefore, in a society like Iran

where addressing sexual and reproductive issues, especially for men, is a cultural taboo, an important solution should be to address the need for men to empathize with their wives when facing sexual and reproductive health problems. Women can present their sexual health and fertility problems to their partner's doctor and/or medical staff for treatment and advice (Kengne-Nde et al., 2021).

Apparently, policymakers should take essential measures such as developing male sexual and reproductive health improvement programs and integrating them with primary health care plans to enhance men's access to such services in society. Such programs should partly include the allocation of suitable opportunities, facilities, and learning environments for men. Moreover, the priorities should also include educating health care providers to provide men with high-quality reproductive health education, care, and services to create a sense of accountability and motivation. It is also essential to improve social awareness through education to attract the support of families, society, and public organizations and non-governmental organizations (NGOs) for men, especially for the vulnerable individuals such as retirees.

Limitations

The limitation in this study was about data collection, which would be strengthened by implementing focus group discussions. However, because of the sensitivity of sexual and reproductive issues in our culture and society and to respect the privacy of individuals, the researcher decided to perform data collection through individual interviews; hence, the lack of group discussions was one of the limitations of this study.

Conclusion

This study indicated that participants faced many challenges related to their reproductive health. Addressing these challenges requires coordinated efforts at multiple levels. Structural interventions may include integrating comprehensive men's sexual and reproductive health services into primary health care systems, improving accessibility and service quality, and promoting equitable distribution of resources. Broader socioeconomic improvements—such as employment stability and financial security—may also indirectly support better health outcomes. They also believed that it would be necessary to motivate the health staff, provide healthy lifestyle education services for men, prevent high-risk behavior, adopt health-seeking behavior, and attract society and family support.

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Ethical Considerations

The protocol of this study has been approved by the Ethics Committee of the Tehran University of Medical Sciences, Tehran, Iran (code number: IR.TUMS.FNM.REC.1397.157). Participation in this study was voluntary and written informed consent was obtained from all participants. Participants were also assured that their information would remain confidential. In addition, before participating in the study, the purpose of the study and the benefits of the study results for improving men's reproductive and sexual health were explained to the participants. This study was conducted in accordance with the principles of the Declaration of Helsinki.

Consent for Publication

Not applicable.

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Declaration of Conflicting Interests

The authors declared no potential conflicts of interest with respect to the research, authorship, and/or publication of this article.

Data Availability Statement

The datasets used and/or analyzed during the current study are available from the corresponding author upon reasonable request.

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Appendix

Semi-Structured Interview Guide for the Sexual and Reproductive Health and Associated Challenges in Iranian Males: A Qualitative Study.

Q1	Interviewer Name	
Q2	Participant ID	
Q3	Interview Date (dd/mm/yyyy)	
Q4	Participant agrees for interview to be digitally recorded	Yes ☐ No ☐
Q5	Time Interview Began (24-hr clock)	_ _ _ _ _ _ _
Q6	Time Interview Ended (24-hr clock)	_ _ _ _ _ _ _

Step 1: Complete Q1–3 above before the interview.
 Step 2: At the beginning of the interview, introduce yourself; thank participant for taking part in the interview.
 Step 3: Read Section A below to participant.
 Step 4: Complete demographic questionnaire
 Step 5: Ask participant permission to record interview; tick appropriate box in Q4 above.
 Step 6: Turn on audio recorder if acceptable, document time interview begins in Q5 above, and conduct interview.
 Step 7: At the end of the interview, thank the participant and ask if she/he has any further questions; document time interview ended in Q6 above.
 Step 8: Ask if the participant is interested in being re-contacted with study results; if yes, document appropriate email or any other contact information. Inform participant that her/his contact information will remain confidential and will not be linked to the study data.

Interviewer: Please read the following to participants at the beginning of the interview.

Section A: Information About This Study

- Hello, thank you for taking time out of your busy schedule to speak with me today. My name is [Name], and I am a _____ from Tehran University Medical Sciences. Do you have time to talk now?
- Before we begin, I'd like to tell you more about the research we're doing and what we will do with the information you tell us.
- With the "Sexual and reproductive health and associated challenges in Iranian males: a qualitative study," we want to gain information about men's understanding of sexual and reproductive health and the challenges associated with it, so that through its results we can help improve sexual and reproductive health in men.
- As explained in the information sheet previously provided to you, participation in this interview is voluntary. You can choose not to answer a question or you can stop the interview at any time.
- With your permission, I would like to audio-record the interview. The audio-recording will be stored on a secure server and destroyed after the findings of this research are published.
- If you do not want the interview audio-recorded, I would like your permission to take detailed notes during the interview.
- The interview will take approximately 45 to 60 min.

- Do you have any questions for me at this point? Information about who to contact if you have questions about the study after our time today, including the Tehran University of Medical Sciences, can be found in the informational sheet.

[If yes, answer the participant's questions then proceed with the completing the demographic form.]

[If no, proceed with the completing the demographic form.]

Is it okay if I turn on the audio recorder now?

[If yes, begin audio recording now.]

[If no] That's okay, I'll take detailed notes as we talk.

Ok, let's get started!

Section B: Participation in the Sexual and Reproductive Health and Associated Challenges in Iranian Males: A Qualitative Study

I would like to begin our discussion by getting a sense of your experience in sexual and reproductive health.

1. Could you tell me your thoughts about reproductive health?
2. Would you tell me how you feel about your reproductive health?
3. What factors do you think affect men's reproductive health?

4. What behavior do you think affects your reproductive health?
5. What do you do to improve your reproductive health?
6. What would help you provide information and services in reproductive health?

For the clarification of explanations, ask further probing questions ("Could you give me an example?" or "Would you explain more?").

Section C: Advantages and Disadvantages of the Sexual and Reproductive Health and Associated Challenges in Iranian Males: A Qualitative Study

Now I'd like to talk to you about your perceptions of the advantages and disadvantages of conducting the "Sexual and reproductive health and associated challenges in Iranian males: a qualitative study"

1. In your opinion, what are the advantages and disadvantages of studying men's perceptions of sexual health and male fertility and the challenges associated with it?
2. How do you think men's reproductive and sexual health problems can be reduced?